



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : إيمان محمد محمود سليم
المحافظة : القليوبية
الحى / المدينة : الخصوص
رقم البطاقة : 27605010101744 رقم التليفون : 01122886191
العمر (تاريخ الميلاد) : 01-05-1976 المهنة : ربه منزل
العنوان : سعيد
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالتدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالتدي (مبادرة)
خطة العلاج : أبحاث أورام مريد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/رمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ. / د.أ.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **أيمن محمد محمود علي**
National ID: **37605010101744**



تقرير طبي

المريض

تحية طبية وبعد،،

بشأن تقرير طبي للمريض.

إهداء للطلب المقدم من

رقم طبي:

رقم:

تشرف بأن أوضح أيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الشغل :
- - - - - 00:00 05-07-2026

وبعد الشك والتموصات والمتابعة وجدته

المرحلة الحالي من يوم غرضه والشيء و يحتاج الى Further investigations and imaging

توقيع :

أحمد محمد علي

التاريخ :

05-07-2026

الطبيب المعالج :
الأستاذ الدكتور



Patient Name : الأسبلاء / ايمان محمد محمود سليم	Request No. : 13005623
Patient No. : 27605010101744	Request Date : 09/04/2025
Path No. : 1246/25	Verification Date : 29/04/2025
Sex : Female	Financial Category : اورام ناز السلام - مياثره
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right wide local excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination:

Five specimens were received:

-Axillary clearance specimen : Two Fibrofatty tissue pieces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.

- Extra medial margin: Fibrofatty tissue piece measured 3x2.5 cm

- Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm

-Right wide local excision specimen measured: 10x9x4 cm with covering skin ellipse measured 9x3.5 cm.

- Tumor extent

o Number: Two (wired) ill defined rubbery greyish white areas.

o Size: 1.5x1x1 cm and 1x0.5 cm.

o Skin involvement: Free grossly

- Distance from margins: 0.5 cm from least lateral margin.

- Frozen section:

-Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.

- Suspicious medial and deep margins

- Free other margins.

Microscopic examination:

- First wired area:

- Histological type: Invasive duct carcinoma.

- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).

Verified by :

Dr.Heba Hasan

Patient Name : الأستاذة / ايمان محمد محمود سليم
Patient No. : 27605010101744
Path No. : 1246/25
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : اورام - دار خديعة - ميثاقه
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Tumor cellularity: 20%.
- Angiolinvasion: Not detected.
- Ductal carcinoma in situ: Not detected.
- Second wired area :
- Only ductal carcinoma component, intermediate grade, (5%).
- There is no evidence of invasive component
- Lymph node involvement: Positive (1/12), with some showed therapy effect.
- Frozen nodes (1/6).
- Paraffin nodes (0/6).
- Resection Margins: Free all resected margins and extra margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearance specimens:

- Invasive duct carcinoma , grade II.
- Tumor cellularity: 20%.
- Free all resected margins and extra margins.
- Lymph nodes: Positive (1/12).
- Pathologic partial response.
- Stage : ypT1b , ypN1.
- RCB : 2.767 , RCB class II
- Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends only on paraffin results.

Prof. Gamal Abdelatty
Dr. Menna Nabil

Verified by :
Dr. Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 2 of 3

Printing By : marwa mohamed yoo



Patient Name	: الأستاذة / سمية بيومي عبد الله محمد	Request No.	: 13002998
Patient No.	: 27807092101901	Request Date	: 05/12/2024
Path No.	: 4780/24	Verification Date	: 15/12/2024
Sex	: Female	Financial Category	: اورام خارج السلام - مياتر
Age	: 46 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 5/12/2024.

Gross Examination:
- 4 grayish white tissue cores up to 1.5 cm, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: Focal.
* Perineural invasion: No.
* Angiolymphatic invasion: No.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with focal DCIS.
For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR. Menna Nabil

Printing Date : 08/04/2025 12:57:08

Page 1 of 1

Printing By : thermal





Name: Mrs. Eman Mohamed Mahmoud Selim

Date: 9/11/2024

ID: 2224069566

Acc: 5047373

Referring Hospital: Azam Dar Al-Salam

Te-99m MDP whole body bone scan

Technique:

Whole body images were acquired 180 minutes after IV injection of 15 mCi of the tracer.

Findings:

Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.

Degenerative changes of the spine are seen.

Arthritic knee changes are noted.

Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandool, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Model No. AM





Patient Name: Eman Mohamed Mahmoud Selim	Date : 25/11/2024
Exam : C.T NECK	Age : 48 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

NON CONTRAST MULTI-SLICE CT SCAN OF THE NECK: -

- Normal appearance of the nasopharynx, oropharyngeal and laryngeal air spaces.
- No glottic, supra or infra glottic mass lesions.
- Intact cartilaginous skeleton of the larynx.
- Normal appearance of submandibular, parotid salivary glands.
- Normal size, enhancement & symmetrical appearance of thyroid glands.
- Normal appearance of the neck muscles & vessels.
- Multiple small deep cervical lymph nodes noted, being subcentimetric in short axis.

Opinion:

- Multiple small deep cervical lymph nodes, likely inflammatory than neoplastic for clinical correlation & follow up.

Dr. Sherif Sameh, M.D.



Patient Name: Eman Mohamed Mahmoud Selim	Date : 24/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 3087/2024	Pat. ID: 77240

**COMBINED BILATERAL BREAST MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND OF BOTH BREASTS**

Indication:

Multifocal right breast pathologically proven malignant masses.

Received NACT

Report:

- Scattered fibro glandular density (ACR B)
- Stable right breast show UOQ multiple spiculated irregular mass lesion, of high density. The area involving the masses measures about 66x65x110mm. Two of which are marked by metallic clip.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions are seen from 10O'clock to 12o'clock, 2mm from skin and nearest one from nipple about 36mm, the largest one measuring about 15x10mm.
- No left suspicious micro-calcifications or spiculated masses.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Still seen one indeterminate right axillary lymph nodes are seen with diffuse cortical thickening about 3.3mm and preserved fatty hilum.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ pathologically proven malignant masses with still noted ipsilateral indeterminate axillary lymph node showing overall stationary course (BIRADS 6).
- Normal study of the left breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Patient ID: 00000	Patient Name: Eman Mohamed Mahmoud Selim
Ref: 00000	09/04/2025 10:47:53 AM Report Date

Patient Name: Eman Mohamed Mahmoud Selim

Date: 09/04/2025

Stereotactic Guided Wire Localization:

Clinical summary: Status post NACT for pre-operative localization of inserted clips.

Technique:

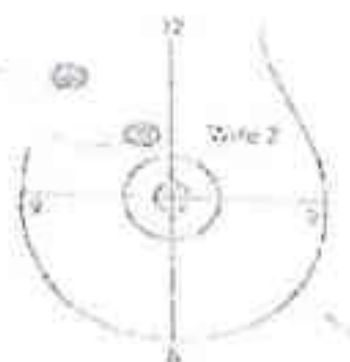
Under complete sterile conditions, 3ml of local anesthetic were administered at the site of concern in the RT breast. Two 21G wires were used to localize the inserted clips.

The wires are located at the right upper outer quadrant.

Wire 1: was inserted perpendicular to the peripherally located inserted clip.

Wire 2: was inserted perpendicular to the anteriorly located clip that was at 5.7 cm from the nipple with the hook seen anterior to the clip by 3.5mm.

Wire 1



Much Obligor
Dr. Sherihan Fakhry, MD.



الجمهورية العربية السورية
مجلس الوزراء

لبنان
محمد محمود سلوم
ابن شكرى سعد
القصور - القلوبية



٢٧٦.٥.١.١.١٧٤٤

100221630



٢٠٢٠/٠٩

٢٧٦.٥.١.١.١٧٤٤

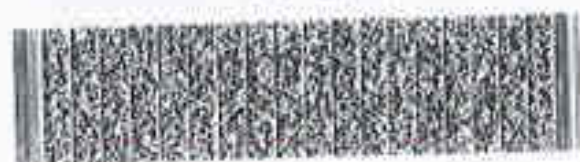
حاصله على ايلوم صليح

ابن شكرى سعد



البطاقة سارية حتى ٢٠٢٧/٠٩/٢٢

١١



Patient's Name:

Iman Mohammed Mahmoud

Date:

2026-06-03

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

مستشفى
جوستاف روسيه
القاهرة

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post right CBS & ALAND, followed by radio therapy, routine follow up study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ, architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.
- **Ultrasound demonstrated** UOQ area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- axillary tail small circumscribed nodule with macrocalcification. No speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound demonstrated** axillary tail small benign looking nodule measuring 6 x 3 mm, no suspicious solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 2): Post-Therapeutic sequelae.

Left breast (BIRADS 2): Axillary tail benign looking nodule.

Recommendation:-

For annual follow up.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Categories & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %
0 %
2 %
30 %

90-100%

NAME: EHSAN MOHAMED MAHMOUD
DATE: 4-1-2026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast:

- Ultrasound demonstrated UOQ posterior third area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla. No pathological infra or supra-cleavicular lymph nodes.
- **Left breast:**
 - Ultrasound demonstrated no solid or cystic lesions.
 - Normal skin/nipple/areola complex.
 - No-specific axillary LNs.

Conclusion:

Right breast (BIRADS 2): Post-therapeutic sequelae.
Left breast (BIRADS 1): Normal study.

Recommendation:
Clear annual follow up.

Best Regards,
Dr. EHSAN MOHAMED MAHMOUD

The above clinical section of breast ultrasound is the final report. Details of procedure used for breast assessment contained within attached report code.

Examiner's recommendation:

BIRADS 01: Non-informative study.

BIRADS 02: Normal study, routine screening.

BIRADS 03: Benign findings, routine screening.

BIRADS 04: Probably benign, for close follow up.

BIRADS 05: Probably malignant, for biopsy.

06: Malignant.

07: Moderate suspicion.

08: High suspicion.

BIRADS 09: Malignant, for biopsy.

BIRADS 10: Pathologically proven malignant.

Selected Clinical Risk

0.1%

0.1%

2.1%

50.1%

BB-100%

Patient Name:

Patient No. 127605010101744

Print No. : 1246/25

Sex
:Female

48 Y

Restoring Doc

Request No. : 13005623

Request Date : 09/04/2025

Verification Date : 29/04/2025

Financial Category : **Capital - Debt**

Department: Out Patient

Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Right breast cancer post NACT.

Nature of the specimen:

Flight wide local excision and auxiliary clearance specimens, collected at 9/4/2025.

Gross Examination:

Five specimens were received:

*Auxiliary clearance specimen : Two Fibroblasty tissue pieces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.

- Extra medial rotation. Fibrofatty tissue piece measured 3x2.5 cm

- Extra deep margin: Fibroblastic tissue placed measured 2.5x2 cm

1000. Voids in the wall of the specimen measured 10.0 mm in diameter and were covered with ellipse measured 193.5 cm.

Tumor extent

○ Number: Two (wired) ill defined rubbery greyish white areas.

Size: 1.5x1x1cm and 1x0.5 cm.

- o Skin involvement: Free grossly

* Distance from margins: 0.5 cm from least lateral margin

— frozen section

Lymph nodes: Free (0/5), six more nodes are left for paraffin examination.

Suspicious medial and deep margins

-Free other margins

Microscopic examination

- First wired area

- Histological type: Invasive duct carcinoma.

* Grade (differentiation + nuclear pleomorphism + mitosis): G1/G3+2+1)

Verified by:

Dr Helga Hasen

Printing Date: 30/04/2025 11:25:19

Page 1 of 3

Printing by a master maintained by



الجمهورية العربية السورية
بureau des affaires indiennes

بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على
سيدنا محمد وآله الطيبين
الطاهرين

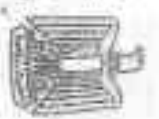


٧ ٧ ٦ . ٥ . ١ . ١ . ١ ٧ ٤ ٤

100221630



٧ . ٥ . ١ . ١ . ١ ٧ ٤ ٤
بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على
سيدنا محمد وآله الطيبين
الطاهرين



٧ ٧ ٦ . ٥ . ١ . ١ . ١ ٧ ٤ ٤
بسم الله الرحمن الرحيم
الحمد لله رب العالمين
والصلاة والسلام على
سيدنا محمد وآله الطيبين
الطاهرين



NAME EMAN MOHAMED MAHMUD
DATE 4-1-2026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study.

Right breast:

- Breastound demonstrated UOD posterior third are of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear right axilla, No pathological infra or supra-clavicular lymph nodes

Left breast:

- Breastound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- No suspicious axillary LNs.

Conclusion:

Right breast (SIRADS 2): Post-therapeutic sequelae.

Left breast (SIRADS 1): Normal study.

Recommendation:

3-6 monthly follow up.

Best Regards,
Dr. Eman Mahmoud, MD

Medical classification of breast lesions according to the American College of Radiology used for breast assessment combined with

Classification	Estimated Cancer Risk
<u>Category A: Recommendations</u>	
1. No US or Mammographic study.	
2. US and Mammographic study, routine screening	0%
3. US and Mammographic study, routine screening	0%
4. US and Mammographic study, routine screening	2%
5. US and Mammographic study, routine screening	5-10%
6. Mild suspicion.	
7. Moderate suspicion.	
8. High suspicion.	
9. US and Mammographic study, for biopsy.	98-100%
10. US and Mammographic study, proven malignant	

Patient Name	Male - years since last birth
Patient No.	: 27605010101744
Pain No.	: 1246725
Sex	: Female
Age	: 48 y
Referring Doc :	

Request No. : 13005523
Request Date : 08/04/2025
Verification Date : 29/04/2025
Financial Category : *مريض - عائله*
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Flight breast cancer post NACT.

Nature of the specimen:

²Light wide local excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination

Five specimens were recaptured:

- Axillary clearance specimen : Two Fibrofatly tissue pieces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.
- Extra medial margin: Fibrofatly tissue piece measured 3x2.5 cm
- Extra deep margin: Fibrofatly tissue piece measured 2.5x2 cm
- Tumor width 10.5 = 10.5 cm specimen thickness 10x9.4 cm with covering 5x1.1 ellipse measured 9x3.5 cm.
- Tumor extent
- Number: Two (wired) ill defined rubbery greyish white areas.
- Size: 1.5x1x1cm and 1x0.5 cm.
- Skin involvement: Free grossly
- Distance from margins: 0.5 cm from least lateral margin.
- Frozen section:
- Lymph nodes: Free (0/6), six more nodes are left for paraffin examination.
- Suspicious medial and deep margins.
- Free other margins.

Microscopic examination:

- First wired area:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): G III(3+2+1)

Verified by:

Dr. Heba Hassan

Printing Date: 20-01-2015 11:25:19

Page 1 of 3

Printing By : mervat mohammed youssef

ECHO REPORT

NAME: محمد بن علي DATE: 29/3/2026
ID NUMBER: / SEX: ♀ AGE:

EF	<u>66%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.5</u>	3.5-5.8cm	AO.R	<u>2.3</u>	3-3.7cm
LVEDSD	<u>2.3</u>	2.4cm	LA	<u>3.6</u>	2-4cm
FS	<u>30%</u>	25-45%	RV		0.7-2.3cm(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RumA
LA and Ao Root Dimensions Normal
Mitral Valve Normal
Aortic Valve Normal
Tricuspid Valve Normal
Pulmonary Valve Normal
Right Side and Systolic Function Normal
Intracardiac Masses and Effusion None

DR.

[Signature]

MUHAMMAD EMLAN MOHAMMED MAHMOUD
DA 73 4-1-2026

Bilateral high resolution breast sonography revealed:

Clinical summary: Post Right CBS and ALAND followed by radiotherapy, routine follow up study

Findings:

Right breast:

- Ultrasound demonstrated UOQ posterior third area of parenchymal disruption underneath the operative scar with no solid, cystic lesions or fluid collections.
 - Thickened skin reaching 5 mm and coarsen trabeculae.
 - Normal nipple/areola complex.
 - Clear right axilla. No pathological infra or supra-clavicular lymph nodes.
- Left breast:
- Ultrasound demonstrated no solid or cystic lesions.
 - Normal skin/nipple/areola complex.
 - No-specific axillary LNs.

Conclusion:

Right breast (BIRADS 2): Post-therapeutic sequelae.
Left breast (BIRADS 1): Normal study.

Recommendation:
2-3 months follow up.

Best Regards,
DR. EMLAN MOHAMMED MAHMOUD

Bibliography: Identification of breast lesions sensitive due to the American College of Radiology used for breast assessment combined with ultrasonography.

Clinical Recommendations:

1-2-3-4-5-6-7-8-9-10-11-12-13-14-15-16-17-18-19-20-21-22-23-24-25-26-27-28-29-30-31-32-33-34-35-36-37-38-39-40-41-42-43-44-45-46-47-48-49-50-51-52-53-54-55-56-57-58-59-60-61-62-63-64-65-66-67-68-69-70-71-72-73-74-75-76-77-78-79-80-81-82-83-84-85-86-87-88-89-90-91-92-93-94-95-96-97-98-99-100-101-102-103-104-105-106-107-108-109-110-111-112-113-114-115-116-117-118-119-120-121-122-123-124-125-126-127-128-129-130-131-132-133-134-135-136-137-138-139-140-141-142-143-144-145-146-147-148-149-150-151-152-153-154-155-156-157-158-159-160-161-162-163-164-165-166-167-168-169-170-171-172-173-174-175-176-177-178-179-180-181-182-183-184-185-186-187-188-189-190-191-192-193-194-195-196-197-198-199-200-201-202-203-204-205-206-207-208-209-210-211-212-213-214-215-216-217-218-219-220-221-222-223-224-225-226-227-228-229-230-231-232-233-234-235-236-237-238-239-240-241-242-243-244-245-246-247-248-249-250-251-252-253-254-255-256-257-258-259-260-261-262-263-264-265-266-267-268-269-270-271-272-273-274-275-276-277-278-279-280-281-282-283-284-285-286-287-288-289-290-291-292-293-294-295-296-297-298-299-300-301-302-303-304-305-306-307-308-309-310-311-312-313-314-315-316-317-318-319-320-321-322-323-324-325-326-327-328-329-330-331-332-333-334-335-336-337-338-339-340-341-342-343-344-345-346-347-348-349-350-351-352-353-354-355-356-357-358-359-360-361-362-363-364-365-366-367-368-369-370-371-372-373-374-375-376-377-378-379-380-381-382-383-384-385-386-387-388-389-390-391-392-393-394-395-396-397-398-399-400-401-402-403-404-405-406-407-408-409-410-411-412-413-414-415-416-417-418-419-420-421-422-423-424-425-426-427-428-429-430-431-432-433-434-435-436-437-438-439-440-441-442-443-444-445-446-447-448-449-450-451-452-453-454-455-456-457-458-459-460-461-462-463-464-465-466-467-468-469-470-471-472-473-474-475-476-477-478-479-480-481-482-483-484-485-486-487-488-489-490-491-492-493-494-495-496-497-498-499-500-501-502-503-504-505-506-507-508-509-510-511-512-513-514-515-516-517-518-519-520-521-522-523-524-525-526-527-528-529-530-531-532-533-534-535-536-537-538-539-540-541-542-543-544-545-546-547-548-549-550-551-552-553-554-555-556-557-558-559-560-561-562-563-564-565-566-567-568-569-570-571-572-573-574-575-576-577-578-579-580-581-582-583-584-585-586-587-588-589-590-591-592-593-594-595-596-597-598-599-600-601-602-603-604-605-606-607-608-609-610-611-612-613-614-615-616-617-618-619-620-621-622-623-624-625-626-627-628-629-630-631-632-633-634-635-636-637-638-639-640-641-642-643-644-645-646-647-648-649-650-651-652-653-654-655-656-657-658-659-660-661-662-663-664-665-666-667-668-669-670-671-672-673-674-675-676-677-678-679-680-681-682-683-684-685-686-687-688-689-690-691-692-693-694-695-696-697-698-699-700-701-702-703-704-705-706-707-708-709-710-711-712-713-714-715-716-717-718-719-720-721-722-723-724-725-726-727-728-729-730-731-732-733-734-735-736-737-738-739-740-741-742-743-744-745-746-747-748-749-750-751-752-753-754-755-756-757-758-759-760-761-762-763-764-765-766-767-768-769-770-771-772-773-774-775-776-777-778-779-780-781-782-783-784-785-786-787-788-789-790-791-792-793-794-795-796-797-798-799-800-801-802-803-804-805-806-807-808-809-810-811-812-813-814-815-816-817-818-819-820-821-822-823-824-825-826-827-828-829-830-831-832-833-834-835-836-837-838-839-840-841-842-843-844-845-846-847-848-849-850-851-852-853-854-855-856-857-858-859-860-861-862-863-864-865-866-867-868-869-870-871-872-873-874-875-876-877-878-879-880-881-882-883-884-885-886-887-888-889-890-891-892-893-894-895-896-897-898-899-900-901-902-903-904-905-906-907-908-909-910-911-912-913-914-915-916-917-918-919-920-921-922-923-924-925-926-927-928-929-930-931-932-933-934-935-936-937-938-939-940-941-942-943-944-945-946-947-948-949-950-951-952-953-954-955-956-957-958-959-960-961-962-963-964-965-966-967-968-969-970-971-972-973-974-975-976-977-978-979-980-981-982-983-984-985-986-987-988-989-990-991-992-993-994-995-996-997-998-999-1000-1001-1002-1003-1004-1005-1006-1007-1008-1009-1010-1011-1012-1013-1014-1015-1016-1017-1018-1019-1020-1021-1022-1023-1024-1025-1026-1027-1028-1029-1030-1031-1032-1033-1034-1035-1036-1037-1038-1039-1040-1041-1042-1043-1044-1045-1046-1047-1048-1049-1050-1051-1052-1053-1054-1055-1056-1057-1058-1059-1060-1061-1062-1063-1064-1065-1066-1067-1068-1069-1070-1071-1072-1073-1074-1075-1076-1077-1078-1079-1080-1081-1082-1083-1084-1085-1086-1087-1088-1089-1090-1091-1092-1093-1094-1095-1096-1097-1098-1099-1100-1101-1102-1103-1104-1105-1106-1107-1108-1109-1110-1111-1112-1113-1114-1115-1116-1117-1118-1119-1120-1121-1122-1123-1124-1125-1126-1127-1128-1129-1130-1131-1132-1133-1134-1135-1136-1137-1138-1139-1140-1141-1142-1143-1144-1145-1146-1147-1148-1149-1150-1151-1152-1153-1154-1155-1156-1157-1158-1159-1160-1161-1162-1163-1164-1165-1166-1167-1168-1169-1170-1171-1172-1173-1174-1175-1176-1177-1178-1179-1180-1181-1182-1183-1184-1185-1186-1187-1188-1189-1190-1191-1192-1193-1194-1195-1196-1197-1198-1199-1200-1201-1202-1203-1204-1205-1206-1207-1208-1209-1210-1211-1212-1213-1214-1215-1216-1217-1218-1219-1220-1221-1222-1223-1224-1225-1226-1227-1228-1229-1230-1231-1232-1233-1234-1235-1236-1237-1238-1239-1240-1241-1242-1243-1244-1245-1246-1247-1248-1249-1250-1251-1252-1253-1254-1255-1256-1257-1258-1259-1260-1261-1262-1263-1264-1265-1266-1267-1268-1269-1270-1271-1272-1273-1274-1275-1276-1277-1278-1279-1280-1281-1282-1283-1284-1285-1286-1287-1288-1289-1290-1291-1292-1293-1294-1295-1296-1297-1298-1299-1300-1301-1302-1303-1304-1305-1306-1307-1308-1309-1310-1311-1312-1313-1314-1315-1316-1317-1318-1319-1320-1321-1322-1323-1324-1325-1326-1327-1328-1329-1330-1331-1332-1333-1334-1335-1336-1337-1338-1339-1340-1341-1342-1343-1344-1345-1346-1347-1348-1349-1350-1351-1352-1353-1354-1355-1356-1357-1358-1359-1360-1361-1362-1363-1364-1365-1366-1367-1368-1369-1370-1371-1372-1373-1374-1375-1376-1377-1378-1379-1380-1381-1382-1383-1384-1385-1386-1387-1388-1389-1390-1391-1392-1393-1394-1395-1396-1397-1398-1399-1400-1401-1402-1403-1404-1405-1406-1407-1408-1409-1410-1411-1412-1413-1414-1415-1416-1417-1418-1419-1420-1421-1422-1423-1424-1425-1426-1427-1428-1429-1430-1431-1432-1433-1434-1435-1436-1437-1438-1439-1440-1441-1442-1443-1444-1445-1446-1447-1448-1449-1450-1451-1452-1453-1454-1455-1456-1457-1458-1459-1460-1461-1462-1463-1464-1465-1466-1467-1468-1469-1470-1471-1472-1473-1474-1475-1476-1477-1478-1479-1480-1481-1482-1483-1484-1485-1486-1487-1488-1489-1490-1491-1492-1493-1494-1495-1496-1497-1498-1499-1500-1501-1502-1503-1504-1505-1506-1507-1508-1509-1510-1511-1512-1513-1514-1515-1516-1517-1518-1519-1520-1521-1522-1523-1524-1525-1526-1527-1528-1529-1530-1531-1532-1533-1534-1535-1536-1537-1538-1539-1540-1541-1542-1543-1544-1545-1546-1547-1548-1549-1550-1551-1552-1553-1554-1555-1556-1557-1558-1559-1560-1561-1562-1563-1564-1565-1566-1567-1568-1569-1570-1571-1572-1573-1574-1575-1576-1577-1578-1579-1580-1581-1582-1583-1584-1585-1586-1587-1588-1589-1590-1591-1592-1593-1594-1595-1596-1597-1598-1599-1600-1601-1602-1603-1604-1605-1606-1607-1608-1609-1610-1611-1612-1613-1614-1615-1616-1617-1618-1619-1620-1621-1622-1623-1624-1625-1626-1627-1628-1629-1630-1631-1632-1633-1634-1635-1636-1637-1638-1639-1640-1641-1642-1643-1644-1645-1646-1647-1648-1649-1650-1651-1652-1653-1654-1655-1656-1657-1658-1659-1660-1661-1662-1663-1664-1665-1666-1667-1668-1669-1670-1671-1672-1673-1674-1675-1676-1677-1678-1679-1680-1681-1682-1683-1684-1685-1686-1687-1688-1689-1690-1691-1692-1693-1694-1695-1696-1697-1698-1699-1700-1701-1702-1703-1704-1705-1706-1707-1708-1709-1710-1711-1712-1713-1714-1715-1716-1717-1718-1719-1720-1721-1722-1723-1724-1725-1726-1727-1728-1729-1730-1731-1732-1733-1734-1735-1736-1737-1738-1739-1740-1741-1742-1743-1744-1745-1746-1747-1748-1749-1750-1751-1752-1753-1754-1755-1756-1757-1758-1759-1760-1761-1762-1763-1764-1765-1766-1767-1768-1769-1770-1771-1772-1773-1774-1775-1776-1777-1778-1779-1780-1781-1782-1783-1784-1785-1786-1787-1788-1789-1790-1791-1792-1793-1794-1795-1796-1797-1798-1799-1800-1801-1802-1803-1804-1805-1806-1807-1808-1809-1810-1811-1812-1813-1814-1815-1816-1817-1818-1819-1820-1821-1822-1823-1824-1825-1826-1827-1828-1829-1830-1831-1832-1833-1834-1835-1836-1837-1838-1839-1840-1841-1842-1843-1844-1845-1846-1847-1848-1849-1850-1851-1852-1853-1854-1855-1856-1857-1858-1859-1860-1861-1862-1863-1864-1865-1866-1867-1868-1869-1870-1871-1872-1873-1874-1875-1876-1877-1878-1879-1880-1881-1882-1883-1884-1885-1886-1887-1888-1889-1890-1891-1892-1893-1894-1895-1896-1897-1898-1899-1900-1901-1902-1903-1904-1905-1906-1907-1908-1909-1910-1911-1912-1913-1914-1915-1916-1917-1918-1919-1920-1921-1922-1923-1924-1925-1926-1927-1928-1929-1930-1931-1932-1933-1934-1935-1936-1937-1938-1939-1940-1941-1942-1943-1944-1945-1946-1947-1948-1949-1950-1951-1952-1953-1954-1955-1956-1957-1958-1959-1960-1961-1962-1963-1964-1965-1966-1967-1968-1969-1970-1971-1972-1973-1974-1975-1976-1977-1978-1979-1980-1981-1982-1983-1984-1985-1986-1987-1988-1989-1990-1991-1992-1993-1994-1995-1996-1997-1998-1999-2000-2001-2002-2003-2004-2005-2006-2007-2008-2009-2010-2011-2012-2013-2014-2015-2016-2017-2018-2019-2020-2021-2022-2023-2024-2025-2026-2027-2028-2029-2030-2031-2032-2033-2034-2035-2036-2037-2038-2039-2040-2041-2042-2043-2044-2045-2046-2047-2048-2049-2050-2051-2052-2053-2054-2055-2056-2057-2058-2059-2060-2061-2062-2063-2064-2065-2066-2067-2068-2069-2070-2071-2072-2073-2074-2075-2076-2077-2078-2079-2080-2081-2082-2083-2084-2085-2086-2087-2088-2089-2090-2091-2092-2093-2094-2095-2096-2097-2098-2099-2100-2101-2102-2103-2104-2105-2106-2107-2108-2109-2110-2111-2112-2113-2114-2115-2116-2117-2118-2119-2120-2121-2122-2123-2124-2125-2126-2127-2128-2129-2130-2131-2132-2133-2134-2135-2136-2137-2138-2139-2140-2141-2142-2143-2144-2145-2146-2147-2148-2149-2150-2151-2152-2153-2154-2155-2156-2157-2158-2159-2160-2161-2162-2163-2164-2165-2166-2167-2168-2169-2170-2171-2172-2173-2174-2175-2176-2177-2178-2179-2180-2181-2182-2183-2184-2185-2186-2187-2188-2189-2190-2191-2192-2193-2194-2195-2196-2197-2198-2199-2200-2201-2202-2203-2204-2205-2206-2207-2208-2209-2210-2211-2212-2213-2214-2215-2216-2217-2218-2219-2220-2221-2222-2223-2224-2225-2226-2227-2228-2229-2230-2231-2232-2233-2234-2235-2236-2237-2238-2239-2240-2241-2242-2243-2244-2245-2246-2247-2248-2249-2250-2251-2252-2253-2254-2255-2256-2257-2258-2259-2260-2261-2262-2263-2264-2265-2266-2267-2268-2269-2270-2271-2272-2273-2274-2275-2276-2277-2278-2279-2280-2281-2282-2283-2284-2285-2286-2287-2288-2289-2290-2291-2292-2293-2294-2295-2296-2297-2298-2299-2300-2301-2302-2303-2304-2305-2306-2307-2308-2309-2310-2311-2312-2313-2314-2315-2316-2317-2318-2319-2320-2321-2322-2323-2324-2325-2326-2327-2328-2329-2330-2331-2332-2333-2334-2335-2336-2337-2338-2339-2340-2341-2342-2343-2344-2345-2346-2347-2348-2349-2350-2351-2352-2353-2354-2355-2356-2357-2358-2359-2360-2361-2362-2363-2364-2365-2366-2367-2368-2369-2370-2371-2372-2373-2374-2375-2376-2377-2378-2379-2380-2381-2382-2383-2384-2385-2386-2387-2388-2389-2390-2391-2392-2393-2394-2395-2396-2397-2398-2399-2400-2401-2402-2403-2404-2405-2406-2407-2408-2409-2410-2411-2412-2413-2414-2415-2416-2417-2418-2419-2420-2421-2422-2423-2424-2425-2426-2427-2428-2429-2430-2431-2432-2433-2434-2435-2436-2437-2438-2439-2440-2441-2442-2443-2444-2445-2446-2447-2448-2449-2450-2451-2452-2453-2454-2455-2456-2457-2458-2459-2460-2461-2462-2463-2464-2465-2466-2467-2468-2469-2470-2471-2472-2473-2474-2475-2476-2477-2478-2479-2480-2481-2482-2483-2484-2485-2486-2487-2488-2489-2490-2491-2492-2493-2494-2495-2496-2497-2498-2499-2500-2501-2502-2503-2504-2505-2506-2507-2508-2509-2510-2511-2512-2513-2514-2515-2516-2517-2518-2519-2520-2521-2522-2523-2524-2525-2526-2527-2528-2529-2530-2531-2532-2533-2534-2535-2536-2537-2538-2539-2540-2541-2542-2543-2544

Referring Doc:	Ref: jms 200 101744
Age	: 48 y
Sex	: Female
Path No.	: 1246/25
Patient Name	: 27605010101744

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : **مستشفى - مستشفى**
Department : Out Patient
Registered by : Mrs Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Eight wide local excision and axillary clearance specimens, collected at 9/4/2025

Gross Examination

Five specimens were recaptured.

Auxiliary clearance specimen: Two Fibrofatty tissue pelices measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.

Extra-medial margin: Fibrofatty tissue piece measured 3x2.5 cm

Extra deep margin: fibrotic tissue piece measured 2.5x2 cm

1000 V and 10 nA scale from positive to negative, coupled with a covering slip ellipse measured 0.35 cm.

Turner, J. 1994.

Number Two (wired) in defined rubbery greyish white areas.

Size: 1.5X1X1cm and 1X0.5 cm.

© Seán involvements: Free grossly

Distance from margins: 0.5 cm from least lateral margin.

Frozen section:

Lymph nodes: Free (0/6), six more nodes are left for paraffin examination.

- Free other margins.

Microscopic examination:

First wired area:

Histological type: Invasive duct carcinoma.

- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)

Verified by:

Dr Heba Hussen

Printing Date: 30/04/2015 11:25:19

Page 1 of 3

Printed by: moses.mohamed@yahoo.com

Patient Name	: John A. Jones MD / MD, PhD
Patient No.	: 27605010101744
Pain No.	: 1246/25
Sex	: Female
Age	: 48 Y
Referring Doc	:

Request No. : 13005623
Request Date : 09/04/2025
Verification Date : 29/04/2025
Financial Category : **المرضى - Out Patient**
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Flight wide local: excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination:

Five specimens were received:

-Auxiliary clearance specimen : Two Fibrofatty tissue pieces measured 5x3.5 cm and 4x3 cm, enclosing 12 nodules up to 2.5x2 cm.

- Extra medial margin: Fibrofatty tissue piece measured 3x2.5 cm

Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm

right wide local excision specimen measured 10.5x4 cm; with covering skin ellipse measured 9x3.5 cm.

- Tumor extent

o Number Two (wired) ill defined rubbery grayish white areas.

o Size: 1.5x1cm and 1x0.5 cm.

o Skin involvement: Free grossly

* Distance from margins: 0.5 cm from least lateral margin.

Frozen section:

-Lymph nodes : Free (0/6), six more nodes are left for section

- Suspicious medial and deep margins

Free other margins.

Microscopic examination:

- First wired area:

-Histological type: Invasive duct carcinoma.

Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)

Verified by:

Dr Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 1 of 3

Printing By : **marwa mohamed youss**

ECHO REPORT

NAME: احمد بن علي

DATE: 29 / 3 / 2026

ID NUMBER: /

SEX: F AGE:

EF	<u>66%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.5</u>	3.5-5.8cm	AO.R	<u>2.3</u>	3-3.7cm
LVEDS	<u>2.3</u>	2-4cm	LA	<u>3.6</u>	2-4cm
FS	<u>30%</u>	25-45%	RV		0.7-2.3cm(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA No RumA

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function TAPE: 2.3

Intracardiac Masses and Effusion

DR.

[Signature]



ERM03672BS45





Ministry of Health and Wellness
Kingdom of Saudi Arabia



MDT form

1- Patient ID
2- Name:

Sex:

3- Medical number:
First:

03231138
Old Med:

03/04/2024

4- ID number: 27603010101744

Patient Age: 49 Y



Family history	Free history	Phone	Age	BMI	Sex	REASON OF MDT
Medical status	None known	Mobile status	0113886191	GROUP	Single	Reason of MDT
CR medications	Chemotherapy	Medication	Medication	Medication	Medication	2023-03-15

Clinical data

Breast Imaging:

1- Breast USQ multiple speculated irregular mass lesion from 18 sectors
2- 12 mm from skin, nearest line from nipple 20 mm target
3- 24 x 17 mm ADOS 5
4- RT intermuscular axillary LN

Mets Workshop:

Free

BRX	ER	PR	Her 2 Neu	SI5H	KIR7
100	0/8	5/8	3	Highly expressed	4
2	0	0	0	0	0

Sonography

CEM: UOQ INDEX LESION 24MM, 2 OTHER LESIONS, IPSILATERAL PATH AXILLA

PATIENT HISTORY

received NACT TCHP x 6 cycles

RT BCS+AD: free margin

YPT 1m largest diameter (1.5CM) LN 1/12

partial pathological response

MDT OUTCOME & PLAN

adjuvant Kadcyla x12
port
Adjuvant AI

SIGNATURE

MEDICAL ONCOLOGY	Signature	RADIOLOGY	Signature
SURGERY	Signature	PATHOLOGY	Signature
Psychiatric	Signature	Medication	Signature

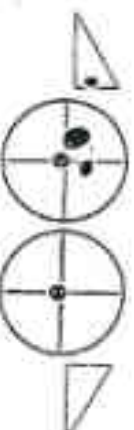


تبريد الماء (الترطيب) للمعدة (الترطيب)

[illegible]

Posterior Angles: 45° & 90°

MI



31/10/2024

References

Breast Implants :

It began by multiple operational breaches: most broken from 10 inches to 12 inches from left, nearest side from middle 35 mm target; one 34 x17 STRAOS 8; one BT (industrial) military LM.

1. 1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100		1. 1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100	
1. 1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100		1. 1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25 26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50 51 52 53 54 55 56 57 58 59 60 61 62 63 64 65 66 67 68 69 70 71 72 73 74 75 76 77 78 79 80 81 82 83 84 85 86 87 88 89 90 91 92 93 94 95 96 97 98 99 100	

Sonomammography

PATIENT HISTORY

CESTM: UOQ INDEX LESION 24MM, 2 OTHER LESIONS, IPSILATERAL PATH AXILLA BONE SCAN

ECHO, LABS

CLIPS INSERTION (2 clips $\rightarrow$ bracketing)

MDT OUTCOME & PLAN

POST-RECEPTIONAL is required

NACT CHEMO TCHP X6 FOLLOWED BY RTBCS

		SIGNATURE	
MEDICAL ONCOLOGY	PHYSICIAN DATE 10/1/11 	RADIOLOGY	DATE 10/1/11 
SURGERY	DATE 10/1/11 	PATHOLOGY	DATE 10/1/11 
Psychiatric	DATE 10/1/11 	NUTRITION	DATE 10/1/11 



Ministry of Health and Family Welfare
Government of Sindh



MDT form

1. Patient ID: 10000000000000000000

Sex:

2. Medical number:

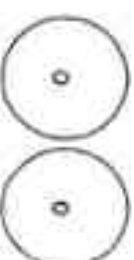
00000000000000000000

Date Recd:

3. ID number:

Patient Age: 40 Y

E M D T



Family history: No history of breast cancer in family
 Menstrual status: Menstrual status: Regular, last period: 10/10/2023
 Co-morbidity: No co-morbidity
 Reason of MDT: Breast lump
 Date of MDT: 05/10/2024

Clinical data

Meta Workshop:

Free

Breast Imaging: Ultrasound: Multiple hypoechoic irregular masses located from 1 to 6 o'clock
 size 2.2 cm from skin, largest one from nipple 2.6 mm largest
 one at inferomedial axillary LN

BR	ER	PR	HER 2	SLISH	HER3
IDC	R/B	S/B	0	0	4%
1	N	0	0	0	0
2	0	0	0	0	0

Sonultramamography

PATIENT HISTORY

FOR CEM THEN REMDT
AND RT AXILARY BIOPSY

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY	PATHOLOGY	NUTRITION
Signature: [Signature]	Signature: [Signature]	Signature: [Signature]	Signature: [Signature]
SURGERY	Signature: [Signature]	Signature: [Signature]	Signature: [Signature]
Psychiatric	Signature: [Signature]	Signature: [Signature]	Signature: [Signature]

Patient Name : السيدة / أمينة محمد حسن
 Patient No. : 27605010101744
 Path No. : 1246125
 Sex : Female
 Age : 48 Y

Request No. : 13005623
 Request Date : 09/04/2025
 Verification Date : 29/04/2025
 Financial Category : السيدات
 Department : Out Patient
 Registered by : Mrs. Mayada

Referring Doc :

Pathology Report

Clinical Data:
 Right breast cancer post NACT, -----

Nature of the specimen:
 Right wide local excision and axillary clearance specimens, collected at 9/4/2025.

Gross Examination:
 Five specimens were received:

- Two Fibrofatty tissue pieces measured 5x2.5 cm and 4x3 cm,
- Axillary clearance specimen : Two Fibrofatty tissue piece measured 3x2.5 cm
- Enclosing 12 nodules up to 2.5x2 cm
- Extra medial margin: Fibrofatty tissue piece measured 2.5x2 cm
- Extra deep margin: Fibrofatty tissue piece measured 2.5x2 cm with covering skin ellipse
- Right wide local excision specimen measured 10.5x7.4 cm with covering skin ellipse
- Right wide local excision specimen measured 9x3.5 cm.
- Tumor extent:
- o Number: Two (wired) ill defined rubbery greyish white areas.
- o Size: 1.5x1x1cm and 1x0.5 cm.
- o Skin involvement: Free grossly
- o Distance from margins: 0.5 cm from least lateral margin.
- Frozen section:
- Lymph nodes : Free (0/6), six more nodes are left for paraffin examination.
- Suspicious medial and deep margins.
- Free other margins.

Microscopic examination:

- First wired area:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation + nuclear pleomorphism + mitosis): G II (3+2+1)

Verified by :
 Dr. Heba Hasan

Printing By : marwa mohar

Printing Date : 30/04/2025 11:25:19

Page 1 of 3

طريق رقم (4) - طبر - (307) - مبنى الطب - مصر الجديدة - القاهرة - مصر
 Route No. (4) - First Floor - Building No. 987 - Nile Corniche - Nasser El Qudous - Cairo, Egypt

CAP

Patient Name : السيدة / Dr. Youssef
Patient No. : 27805010101744
Path No. : 3835/24
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13007115
Request Date : 08/10/2024
Verification Date : 24/10/2024
Financial Category : Out Patient
Department :
Registered by : marwa mohammed yousa

Pathology Report

Clinical Data: Known case of right breast cancer, enlarged right axillary lymph nodes.
Nature of Specimen: Tru-cut biopsy, collected at 08/10/2024.

Gross Examination:

- Multiple grayish white tissue cores up to 0.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for GATA3, ER, PR, Her2, GATA3 clone (L50-823), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- Histopathological examination of sections prepared from the lymph node biopsy received reveals complete effacement of the normal nodal architecture. There is replacement by malignant epithelial neoplasm formed of sheets and irregular tubules of malignant epithelial cells. The malignant cells exhibit moderate degree of anaplasia
- IPT (Positive control showed appropriate reactivity):
 - GATA3: Strong diffuse nuclear reaction of tumor cells.
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
 - PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
 - Her2: Strong complete membrane reaction in >10 % of tumor cells (score 3+).
 - Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

Known case of right breast cancer with enlarged right axillary lymph nodes, Tru-cut biopsy.
Metastatic adenocarcinoma of breast origin.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).

Verified by :

DR. Nermeen Mohamed

Printing Date : 24/10/2024 11:21:54

Page 1 of 2

Printing By : marwa mohammed yousa

د. مهنى ابراهيم سامى
مكتوبه الامامه المتخصصه
امام بكليه طبى المنصر المتعلق



د. ابراهيم علي الشبي
مكتوبه الامامه المتخصصه
امام بكليه طبى المنصر المتعلق

Name: Mrs. Eman Mohamed Mahmoud Selim
Date: 9/11/2024
Referring Hospital Awram Dar Al-Salaam.

ID:2224069566

Acc.:5047373

Tc-99m MDP whole body bone scan

Technique:

- Whole-body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

No scintigraphic evidence of metastatic osseous deposits.

Truly yours

Prof. Dr. Ahmed Kandeel, MD

Prof. of Nuclear Medicine, faculty of medicine, Cairo University

Printed by AR



مملكة البحرين
وزارة الصحة والرفاه الاجتماعي
لدى قسم صحة المرأة المصرية

الطبيب المعالج
د. محمد
م. م. م.

MDT FORM

9 / 3 / 2025 G C. C. 33

Name	د. محمد	MRN	18524155A		
Age	49y	ID	1004900-009		
Menopausal status	Post	Phone	1090001444		
NRS SCORE		Marital status	Divorced		
BMI		Family history	None		
CO morbidity	DM on oral tabs	REASON OF MDT	Post		
Clinical Data					
BX: case of RT ADC involving at ACS+AD, received Noddy CH 9/4/2025	R	T			
ER: 8/8	PR: 3/8	HER 2: +3	Ki67: 4		
Sonomammography: free margins post-op pathology: free margins at extra margins LVI 1/12 pathologic partial response T1bN1					
Mets Workup: free					
Recommendations:					
MDT OUTCOME & PLAN					
SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name	: اسم المريض / اسم العائلة / اسم المريض	Request No.	: 13005623
Patient No.	: 27605010101744	Request Date	: 09/04/2025
Path No.	: 1246/25	Verification Date	: 29/04/2025
Sex	: Female	Financial Category	: مرضى - مرضى
Age	: 48 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

- Tumor cellularity: 20%.
- Angioinvasion: Not detected
- Ductal carcinoma in situ: Not detected
- Second wired area :
- Only ductal carcinoma component, intermediate grade, (5%).
- There is no evidence of invasive component
- Lymph node involvement Positive (1/12), with some showed therapy effect.
- Frozen nodes (1/6).
- Paraffin nodes (0/6).
- Resection Margins: Free all resected margins and extra margins.
- Histologic response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, wide local excision and axillary clearance specimens;

-Invasive duct carcinoma , grade II.

-Tumor cellularity: 20%.

-Free all resected margins and extra margins.

-Lymph nodes: Positive (1/12)

-Pathologic partial response.

-Stage : ypT1b , ypN1

-RCB : 2.767 , RCB class II

Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends only on paraffin results.

Prof. Gamal Abdelatty

Dr. Menna Nabil

Verified by :

Dr. Heba Hasan

Printing Date : 30/04/2025 11:25:19

Page 2 of 3

Printing By : marwa mohamed youss





Patient Name : **المنيرة محمد**
Patient No. : 27605010101744
Path No. : 3835/24
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13007115
Request Date : 09/10/2024
Verification Date : 24/10/2024
Financial Category : **مريض** - Out Patient
Department :
Registered by : marwa mohamed youss

Pathology Report

- Her2: Positive (score 3+).

- Ki67: Positive in 10% of tumor cells.

• Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none, 1, weak, 2, intermediate, and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013, IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells

Signature
Dr. Wajaa Ghonaim,
Dr.Nermiin Mohamed

Verified by :
DR.Nermoen Mohammed

Printing Date : 24/10/2024 11:31:54

Page 1 of 2

Printing By : marwa mohamed youss





Patient Name: Erian Mohamed Mahmoud		Date : 07/08/2024	
Sex: M		Age : 48 Year(s)	
Exam : MAMMOGRAPHY BOTH		Pat. ID: 77240	
Medical ID: 27605010101744			

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast palpable lump.

Positive family history (her mum).

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ multiple spiculated irregular mass lesion, of high density.
- Complementary ultrasound revealed right breast multiple irregular spiculated hypoechoic mass lesions are seen from 10o'clock to 12o'clock, with internal vascularity detected within on color Doppler, seen 2mm from skin and nearest one from nipple about 36mm, the largest one measuring about 24x17mm, consistent to the palpable lump.
- No left solid or cystic mass lesion.
- Normal skin thickness, and nipple-areola complex of the left breast.
- One indeterminate right axillary lymph nodes are seen with diffuse cortical thickening about 3.3mm and preserved fatty hilum.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ multiple malignant looking mass lesions (BIRADS 5) for histopathological correlation.
- One right indeterminate axillary LNs.
- Benign findings left breast (BIRADS 2)

Signature Dr. Nehal Hamdy

07/08/2024 1:41 PM

- No speculated masses, asymmetry, distortion or suspicious calcifications.
- Recombined (post contrast) images show: no pathological uptake.
- Ultrasound demonstrated UOQ 3.00 oval isoechoic lesion measures 7x3mm.
- No cystic masses.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast : 10-12:00 multifocal malignant process with finding as described (BIRADS6) .

Left breast : UOQ probably benign non enhancing mass lesion (BIRADS3) for 6 months follow up.

DR. Sharmistha Karmacharya

Patient Name	: محمد بن عبد الله بن محمد
Patient No.	: 27707062103141
Path No.	: 173325
Sex	: Female
Age	: 47 Y
Referring Doc. :	

Recuest No.	: 13004542
Request Date	: 13/05/2025
Vefication Date	: 30/06/2025
Financial Category	: ٠٠ - مريض
Department	: Out Patient
IHC No.	:
Registered by	: marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left breast cancer (Post-neoadjuvant therapy).

Nature of Specimen:

Left modified radical mastectomy.

Techniques

- Sections of the unstained slides, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

- Her2 clone (4B5)

- IPT:

- Her2: Negative membrane reaction in tumor cells (score 0)

Diagnosis:

Left breast cancer (Post-neoadjuvant therapy). Left modified radical mastectomy.

Residual invasive lobular carcinoma, Grade II, with focal LCIS

IPT:

- Herz: Negative (score 0).

- Scoring system: "Herz"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and

Verified by

Dr. Duaa Mohamed





دكتور محمد بن عبد الله
مستشار طبي
مستشار طبي
مستشار طبي
مستشار طبي

1000

Name: Doaa Fathy

Echo report

Date: 11/5/2025

age: 23Y

EF	71%	54-74%	PWd	0.8	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO R	2.5	2-3.7cm
LVEDS	2.9	2-4cm	LA	2.4	2-4cm
FS	41	25-45%	RV		0.7-2.3cm(supine)
IVSd	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

Normal LV internal dimensions and systolic function (EF=71%) by M-mode.

Reversed mitral inflow pattern.

No resting SWMA could be detected.

Normal LA size.

Mitral Valve: Normal valve with trivial MR.

Aortic valve: Normal valve and flow.

Tricuspid valve: Normal valve with trivial TR.

Right side and systolic function: Normal by TDI.

No intracardiac masses or thrombi.

Normal pericardium with no pericardial effusion.

Normal IVC diameter and collapsibility.

Conclusion:

- Normal LV internal dimensions and systolic function.
- Left ventricular diastolic dysfunction grade I.

Dr Eman Kassem, MD
Cardiology



Presidential Initiative for Women's Health - Pathology Lab
 الطباعة الإلكترونية لخدمة المرأة - معمل الفحوصات النسائية



Patient Name	: اسم المريض / Patient Name	Request No.	: 13004542
Patient No.	: 27707060103141	Request Date	: 13/05/2025
Path No.	: 1733/25	Verification Date	: 29/05/2025
Sex	: Female	Financial Category	: مريض - Patient
Age	: 47 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youssef

Pathology Report

Clinical Data: Left breast cancer post NACT.

Nature of the specimen: Left breast & Left axilla (MRM)(Posi NACT)

Gross Examination:

- Procedure: Left MRM measured 17x13x3 cm with covering skin ellipse measured 17x10cm.
- Localization: LOQ
- Tumor extent: A greyish white tumour bed site
- Size: 1.8 x1.5 x1.5 cm.
- Skin involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 2 cm away from deep margin
- Axilla: Fibrofatty tissue piece measured 5x5x2 cm entangling 13 nodules up to 1.5x1 cm
- Microscopic examination:
- Histological type: Residual invasive lobular carcinoma. Cellularity 30%
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Lobular carcinoma in situ: Focal
- Lymphovascular emboli: Positive
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive L.Ns (5/13)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic partial response

Diagnosis:

- Left MRM (post NACT):
- Residual invasive lobular carcinoma grade II with focal LCIS
- Free deep margin
- Free nipple and areola
- Positive L.Ns (5/13)

Verified by:

Dr. Heba Hasan

Printing Date : 05/06/2025 12:17:59

Page 1 of 1

Printing By : marwa mohamed youssef



Imaging report



Name : Mrs. Doaa Farhy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

F-18 FLUORODEOXYGLUCOSE (FDG) PET/CT OF THE WHOLE BODY:

◆ Clinical History:

- Metastatic workup of pathological proven left breast cancer.

◆ Procedure:

- Dose/Radiopharmaceutical: 8 mCi F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Blood glucose level: mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- A diagnostic post-contrast CT examination of the same regions was performed after IV administration of non-ionic contrast.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.

◆ Findings

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Imaging report



كايرو سكان
CAIRO SCAN
Scanning & USS

Name : Mrs. Doaa Farhy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

- No metabolically active or pathologically enlarged axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal tracer biodistribution is noted at the myocardium and great vessels.

Abdomen and Pelvis:

- Liver is of average size, showing homogenous texture and smooth outline.
- No hepatic focal lesions or areas of abnormal enhancement or metabolic activity.
- No biliary radicles dilatations could be noted.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC as well as pelvic organs
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.
- No pathologically enlarged or metabolically abdominal or pelvic lymph nodes.
- No ascites.

Musculo-skeletal:

- Solitary hypodense bony lesion is seen occupying D5 vertebral body with SUV max 4.



- Multiple tiny dense sclerotic foci are seen at the left iliac bone, few lumbo-sacral vertebrae mainly at D12 vertebral body.



Patient Name: Doaa Far'hy Hussein Ibrahim	Date : 22/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LOQ spiculated irregular mass lesion with overlying microcalcifications
- Complementary ultrasound revealed :
 - **Left breast:**
 - Left breast LOQ at about 4 o'clock irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, 1.5cm from the nipple, 0.3cm from the overlying skin, measuring 2.4x1.2 cm, consistent to the palpable lump.
 - Another two satellite lesions seen at 5 o'clock measured 12x9mm and 5-6 o'clock measured 7x4mm
 - Single pathological left axillary lymph node showing muffled hila, measuring 1.1x1.6cm, with cortical thickness reaching 5mm.
 - Free left supra and infraclavicular regions
 - **Right breast:**
 - No solid or cystic mass lesions
 - Nonspecific right axillary LN.

Opinion:

- Left breast LOQ multifocal malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Basma Mohamed
22/03/2025 2:39 PM

- tiny hypodense hepatic focal lesions, being less appreciated in delayed phase, the largest is seen at segment VIII measuring 8 mm. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Slightly bulky left adrenal gland.
- Normal appearance of the pancreas, spleen, kidneys, right adrenal, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Few prominent para-aortic lymph nodes.
- No abdominal or pelvic masses or collections.
- The revised bone window revealed:
 1. Left iliac wing, left upper femoral shaft, D12 vertebral body, left D9 transverse process, left 4th rib and sternal tiny sclerotic foci likely bone islands
 2. D4 vertebral body lucent lesion measuring about 10mm.
 3. L5 subchondral sclerotic focus measuring 9 mm.
 4. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

Known case of left breast cancer showing:

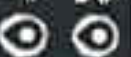
- Few hepatic small focal lesions as described too small to characterize for follow up.
- No evidence of pulmonary metastatic deposits.
- L5 sclerotic focus for better bone scan.

Reported By,

Dr. Yara Alaa, MSc

Dr. Mohamed Mohsen, MD

19-Sep-2024 07:30 PM





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - مختبر الأمراض



Patient Name
Patient No. 27707060103141
Path No. 2964/24
Sex Female
Age 47 Y
Referring Doc.

Request No. 13002328
Request Date 17/08/2024
Verification Date 08/09/2024
Financial Category
Department Out Patient

Registered by marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Marwa Mahmoud.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 08/09/2024 16:25:21

Page 2 of 2

Printed By : marwa mohamed Youssef



طريق رقم (4) - الجسر المعلق - الجيزة - (PRT) - مستشفى الجيزة - القاهرة - مصر
Kousa No. (4) - First Floor-Building No. 987- Nile Corridor - Mid El Qadima - Cairo, Egypt

BQHEYA

(Baqheya Department)

بقرية

قسم الأشعة

PATIENT NAME: DOAA FATHY HUSEN IBRAHIM

GENDER: Female

DATE : 19-Sep-2024 11:28 AM

PATIENT ID: 10124249274

AGE: 47 y

ORDER: 3041678,3041679

**POST CONTRAST MULT-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE
LIVER**

TECHNIQUE:

- Multi-slice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 80 ml.

FINDINGS:

1-CHEST

- Left LOQ and retroareolar asymmetrical and enhancing parenchymal densities.
- Multiple pathological left level I, II and III axillary lymph nodes, the largest measures about 26x10mm.
- Few prominent left supraclavicular lymph nodes, the largest measures about 7x4mm.
- Few right middle and lower lung lobe calcified nodules likely post-granulomatous sequel.
- Left lower lung lobe thin atelectatic band.
- No sizable other pulmonary nodules, pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Right thyroid lobe hypodense nodule for US correlation.

2- ABDOMEN AND PELVIS

- The liver is enlarged with homogenous CT texture showing bi-lobar few

PATIENT: DOAA FATHY HUSEN IBRAHIM / 10124249274 / Female / 47 y / 19-Sep-2024 11:28 AM / 3041678,3041679 / 10124249274 / 11/28/2024 11:28 AM



www.baqheya.org



16602

قسم الأشعة - باقرية

الطبيب: د. أحمد محمد عبد الله

11/28/2024 11:28 AM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة
مركز التخصص في أمراض السرطان
مستشفى دار السلام
إدارة الأشعة

Patient Name: Doaa Farhy Hussein Ibrahim	Date : 08/09/2024
Exam : C.T ABDOMEN & PELVIS	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:

- Pathologically enlarged left axillary lymph nodes level I, II and III as well as left subpectoral, The largest at level I measuring 11 x 23 mm.
- Multiple discretely prominent right axillary lymph nodes are also noted.
- Middle lobe tiny calcific nodule measuring 3 mm. Otherwise, clear both lung fields. No sizable pulmonary nodules and no masses, consolidation or cavitory lesions.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Mildly enlarged liver with homogenous CT texture. No definite focal lesions on non contrast basis or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Imaging report

Name : Mrs. Doaa Far'hy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

F-18 FLUORODEOXYGLUCOSE (FDG) PET/CT OF THE WHOLE BODY:

◆ Clinical History:

- Metastatic workup of pathological proven left breast cancer.

◆ Procedure:

- Dose/Radiopharmaceutical: 8 mCi F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Blood glucose level: mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- A low-dose non-contrast CT images for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- A diagnostic post-contrast CT examination of the same regions was performed after IV administration of non-ionic contrast.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed

◆ Findings

Head and Neck:-

- The brain appears normal on CT, with no abnormal focal metabolic activity detected.
- No pathologically enlarged or metabolically active cervical lymphadenopathy.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx, thyroid and salivary glands.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.

Imaging report



كايرو سكان
CAIRO SCAN
Imaging & Lab

Name : Mrs. Doaa Fathy Hussein Ibrahim

Date : 01/10/2024

Code : 3098456

- No metabolically active or pathologically enlarged axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- Normal tracer biodistribution is noted at the myocardium and great vessels.

Abdomen and Pelvis:

- Liver is of average size, showing homogenous texture and smooth outline.
- No hepatic focal lesions or areas of abnormal enhancement or metabolic activity.
- No biliary radicles dilatations could be noted.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta and IVC as well as pelvic organs
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.
- No pathologically enlarged or metabolically abdominal or pelvic lymph nodes.
- No ascites.

Musculo-skeletal:

- Solitary hypodense bony lesion is seen occupying D5 vertebral body with SUV max 4.



- Multiple tiny dense sclerotic foci are seen at the left iliac bone, few lumbo-sacral vertebrae mainly at D12 vertebral body.



مراجعة رئيس الجمهورية
لحالة صحة المرأة المصرية

مراجعة
1-0

MDT FORM

/ / 20

G

C: C

Name	د. س. م. م. م.	MRN	123456789
Age	44	ID	123456789
Menopausal status	Postmenopausal (Unknown)	Phone	01012345678
NRS SCORE		Marital status	
BMI		Family history	15/11/1990
CO morbidity		REASON OF MDT	
Clinical Data			
BX: ILC G1	R	T1N1M0	
ER: 8/8	PR: 6/8	HER2: 800	KI67: 15%
Sonomammography: 1st LOQ CX8mm			
Mets Workup: PET/CT & FNA of DVS Lm			
Recommendations:			
MDT OUTCOME & PLAN			
SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Nutrition	Psychiatric		



Patient Name: Doaa Farhy Hussein Ibrahim	Date : 22/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 3055/2024	Pat. ID: 78865

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral Breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows LOQ spiculated irregular mass lesion with overlying microcalcifications
- Complementary ultrasound revealed :
 - Left breast:
 - Left breast LOQ at about 4 o'clock irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, 1.5cm from the nipple, 0.3cm from the overlying skin, measuring 2.4x1.2 cm, consistent to the palpable lump.
 - Another two satellite lesions seen at 5 o'clock measured 1.2x0.9m and 5-6 o'clock measured 7x4mm
 - Single Pathological left axillary lymph node showing muffled hila, measuring 1.1x1.6cm, with cortical thickness reaching 5mm.
 - Free left supra and infraclavicular regions
- Right breast:
 - No solid or cystic mass lesions
 - Nonspecific right axillary LN.

Opinion:

- Left breast LOQ multifocal malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological left axillary LNs.
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Basma Mohamed

22/03/2025 2:39 PM



Presidential Program for Women's Health
Ministry of Health and Population



البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

As-patient ID:
1- Name: **فاطمة حسن الزامل**

Sex:

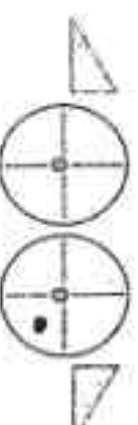
2- Medical number:
Female

8330862
DOB Month:

DOB Year:
2025-2026

2-ID number: 37707001003141

Patient Age: 48 Y



Family history: **None**
Smoking status: **non-smoker**
Ca morbidity: **No report**

Phone: **01773519751**
MBS Status: **not done**
Mammogram status: **not done**

SHI: **SHI**
GROUP: **GROUP**
Initial STAGE: **Initial STAGE**

Reason of MDT: **Reason of MDT**
Date of MDT: **2025-05-10**

Clinical data

Breast Imaging:
CT LUG BWS MAM
LT AXILARY IN STRADS III
RT breast BIRADS III

Mets Workup:
pet ct FDG avid
DVS lesion

ER	ER	PR	HER 2 Neu	SI-SH	KI67
ILC	B/B	6/8	0	SI-SH	15
no 204 (99%)	no 122 (99%)	no 122 (99%)	no 1 (99%)	no 122 (99%)	

T	N	M
ct3	3b	Solitary DVS
multicentric		

Sonomammography

PATIENT HISTORY

Initially invasive lobular carcinoma ,rt breast BIRADS III
received 4 AC->4 taxotere with marked regression
csmilt breast BIRADS 6 residual area of non mass enhancement at the LoQ 2/4 octock

MDT OUTCOME & PLAN

FOR LEFT MAM THEN SBRT ON DS THEN HORMONAL TTT

SIGNATURE

MEDICAL ONCOLOGY	PHYSICIAN SIGNATURE	RADIOLOGY	PATHOLOGY	REMARKS
SURGERY	H. Kalo			
Psychiatric	Dr. H. Kalo	Nutrition		